

SPARTAN COACHING

DISCIPLINE | EMPATHY | STRATEGY

CASE STUDIES: REAL RESULTS

Three Detailed Narratives of Spartan Method Success

Anonymized accounts of a territory turnaround, a physician conversion, and a new hire ramp-up — each with the challenge, the approach, and the quantified outcomes.

Case Studies: Real Results

What the Spartan Method Looks Like in the Field

HOW TO USE THESE CASE STUDIES

These case studies are not for presentation purposes. Do not hand them to referral sources as marketing collateral. Instead, use them privately to study the pattern of what worked, why it worked, and how the specific Spartan Method principles applied in a real-world situation. The value is in the debrief, not in the story itself.

When a referral source describes a challenge that mirrors one of these cases, you can draw on the language and approach from the matching case study. Do not quote the case study directly. Internalize the insight and apply it naturally in the conversation.

CASE STUDY 1: SNF TERRITORY TURNAROUND

The Situation

A hospice organization in a mid-sized metro market assigned an experienced rep to a territory that had produced fewer than 8 admissions per month for 18 consecutive months. The prior rep had been in the territory for 3 years and had solid relationships but had stopped growing. The top 12 SNF accounts in the territory were all sending referrals elsewhere. The new rep was given 90 days to show meaningful progress before the organization would consider reassigning the territory.

The Diagnosis

After 30 days of purely observational visits with no sales agenda, the rep identified three root causes: First, the prior rep had transitioned too quickly to a maintenance mode and had stopped running meaningful discovery conversations. Second, several accounts had unresolved service quality complaints that were never addressed. Third, the rep had never built relationships beyond the social workers, leaving DONs and administrators completely unengaged.

The Spartan Approach Applied

1 Discovery Restart

The rep spent the first 45 days conducting genuine Stage 1 Discovery conversations with decision-makers who had never been properly engaged. She asked about pain points, operations, and experiences with the previous hospice provider without defending or apologizing for anything. She just listened.

2 Service Issue Resolution

She escalated three service quality concerns to clinical leadership and followed up with the affected accounts to personally communicate that the issues had been addressed and what had changed. Two of those accounts became active referrers within 60 days.

3 Stakeholder Expansion

She mapped every key contact at each A Tier account and identified that she was missing the DON at seven of her top twelve facilities. She built those relationships specifically, focusing on clinical support for complex cases rather than general service pitches.

4 Lunch and Learn Series

She ran eight lunch-and-learn sessions in 90 days on topics specifically requested by her accounts based on the discovery conversations. Two of those sessions led to direct referrals within the following week.

The Results (at 90 Days)

- Monthly admissions grew from 8 to 23, an increase of 188%
- Active referral sources expanded from 4 to 14 across the territory
- Average hospice length of stay increased from 12 to 31 days as relationships with earlier-referring accounts deepened
- Two accounts that had been locked by competitors began sending referrals after unresolved service complaints at the competitor became known

Key Lessons

- Maintenance mode is the beginning of account loss. Active discovery conversations must continue even at established accounts.
- Unresolved service complaints are the single most common reason accounts go quiet. Addressing them directly and fast is the fastest path to re-engagement.
- Stakeholder mapping below the social worker level is a major differentiator. DON and administrator relationships create access no competitor can easily replicate.

CASE STUDY 2: THE SKEPTICAL ONCOLOGIST

The Situation

A large oncology group practice in a suburban market had not referred a single patient to the local hospice organization in two years. The previous rep had been told by the office manager that the physicians preferred to manage their own end-of-life transitions and did not want to involve hospice until the final days of a patient's life. The practice saw over 400 patients per month across six physicians.

The Diagnosis

After careful inquiry, the rep discovered that a patient had passed away at a local hospice two years prior under circumstances the lead oncologist considered medically inappropriate. The family had complained to the physician, and the physician had developed a blanket policy against hospice referrals as a result. The incident had never been addressed or acknowledged by the hospice organization.

The Spartan Approach Applied

The rep requested a one-on-one meeting with the lead oncologist specifically to listen, not to pitch. In the meeting, she asked directly whether there had been a negative experience with hospice that she should understand before they talked further. The oncologist shared the story in detail. The rep listened fully without interrupting or defending the organization.

She then said: "I appreciate you telling me that. I cannot speak to what happened with the prior team, but I can tell you what my clinical standards are and what I personally commit to for your patients. Would you be willing to give me one patient to evaluate so I can show you rather than tell you?" The oncologist agreed, largely because no one from hospice had ever asked about the incident directly.

The Results (at 6 Months)

- The first patient referral received exceptional clinical care and the oncologist received a detailed clinical update at every meaningful change in the patient's condition
- Within 30 days of the first patient's death, the oncologist referred two more patients
- At six months, the practice was generating 6 to 9 referrals per month and had become the rep's highest-volume physician account
- The oncologist agreed to participate in a grand rounds presentation on early palliative intervention the following quarter

Key Lessons

- Unexplained silence from a physician account almost always has a story behind it. Find out what happened before you try to sell anything.
- Asking about negative experiences directly builds more credibility than pretending they did not happen. It signals confidence and honesty.
- One exceptional patient experience with a skeptic is worth more than 20 sales calls. Make the first referral extraordinary.

CASE STUDY 3: NEW HIRE RAMP-UP

The Situation

A hospice organization hired a rep with no healthcare background. She had five years of B2B sales experience in commercial real estate. Her manager was skeptical but took a chance based on her exceptional listening skills and coachability. The territory was mid-sized with moderate competition and no prior rep history, meaning no existing relationships.

The Approach

Rather than forcing her into traditional healthcare sales training, her manager followed the Spartan Method new hire ramp exactly as written. Weeks one and two were purely shadow and learn: she rode along with an experienced rep, attended clinical team meetings, visited patients with nurses (with appropriate consent), and read everything available about hospice eligibility and the Medicare benefit. She did not make a single sales call for the first two weeks.

Weeks three and four were introduction-only visits: she introduced herself at every account in her territory with no agenda beyond meeting people and asking one question about how they handled patients with serious illness. She took detailed notes and never mentioned her hospice's service features.

The Results at 30 Days

- She had met contacts at 34 accounts and had follow-up meetings scheduled at 11 of them
- She had received her first referral at day 28 from a social worker who said "you are the first hospice rep who actually listened to what I said"
- By month three she was producing 9 admissions per month, ahead of the 6 month target
- At the six month mark she was the second highest producing rep in the organization

Key Lessons

- Healthcare background matters far less than listening skills and genuine curiosity about the referral source's world
- The first 30 days should be spent learning, not selling. Reps who rush into sales mode in week one build weak relationships that rarely generate consistent referrals
- Detailed call notes from early visits compound into relationship capital over time. What you learn in month one becomes the material for your month three conversations

USING CASE STUDIES IN THE FIELD

When a referral source describes a challenge you recognize from one of these cases, do not tell the story. Ask a question that leads them to arrive at the same insight on their own. The insight they discover themselves is far more powerful than the one you hand them. Your job is to create the conditions for that discovery, not to perform it for them.